
ENGAGEMENT KICKOFF / THE WORK IS UNDERWAY

Haven Paediatric Centre

Diagnostic Audit: how we work, what we examine, what we need

The engagement is mobilised and the audit has begun. This is the full picture: your team, our approach, every area we are digging into, the information we will need, and the surveys already live.

Engagement Lead: Tito Ipinmoye, Director of Product & Strategy

Partner oversight: Dr Debo Odulana, Founding Partner

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This is a go: the engagement is live

Mobilisation is complete, your Consult for Africa team is in place, and data collection has already started. Here is exactly where things stand today.

Mobilised

Engagement commenced and active; four-week diagnostic audit begun

Team in place

Engagement lead and partner oversight assigned and ready

Surveys live

Staff and patient experience surveys already collecting responses

Quick wins started

Crash-cart standard and receivables recovery from week one

Nothing here is theoretical. The work has started, and this deck shows you the whole plan.



Led by Tito Ipinmoye, with partner oversight from Debo Odulana



ENGAGEMENT LEAD

Tito Ipinmoye

Director of Product & Strategy · Engagement Lead

A clinician by training and a healthcare executive by practice, with nearly a decade transforming hospitals, health systems and health-tech ventures across Europe and Africa.

She pairs frontline clinical insight with the strategic and operational discipline to build resilient, high-performing healthcare organisations: operational diagnostics, clinical governance, financial sustainability, process redesign and digital innovation.

Also Chief Product Officer at Cooked Indoors; completing an Executive MBA at ESADE Business School, Barcelona.

For Haven she owns day-to-day delivery of the audit end to end: stakeholder engagement, operational and clinical assessment, diagnostics, analysis and the recommendations.



PARTNER

Dr Debo Odulana

Founding Partner · Partner oversight

Founding Partner of Consult for Africa, a doctor and healthcare entrepreneur who has built and advised health ventures across Nigeria and beyond.

On Haven he provides senior partner oversight and strategic guidance, staying close to the board and the shape of the engagement while Tito leads delivery.

The model is deliberate: a dedicated engagement lead on the ground, backed by partner-level judgement, so Haven gets both intensity and seniority.

A strategic diagnostic, not an operational checklist.

We read Haven through one lens: alignment. Strong facilities get into difficulty when how they are built to run drifts out of line with where they want to go. Our job is to find those gaps and close them in the right order.

Culture is the multiplier

Build the routines, standards and incentives first; safe care, healthy margins and growth follow.

Evidence over impression

Every finding grounded in records, observation and the voice of each stakeholder.

Collaborative, not ambush

We work alongside your team as partners in the findings, never to catch anyone out.



A complete, MECE picture: every domain, no gaps, no overlaps

We assess the whole facility across ten domains, grouped by the enablers that make care safe and the business sustainable. Nothing important is left out; nothing is double-counted.

1

Demand &
Market

2

Access &
Patient Flow

3

Clinical Care
& Governance

4

Pharmacy &
Supply Chain

5

Revenue
Cycle

6

Finance &
Capital

7

People &
Organisation

8

Infrastructure
& Utilities

9

Digital &
Technology

10

Governance
& Compliance

Each domain is examined through six cross-cutting lenses (culture, cash, data, risk, stakeholder voice, scalability), a genuinely MECE structure.



Five perspectives, each with a benchmark-grade instrument

A hospital has more customers than patients. We measure the experience of everyone with a stake, using validated tools so results can be benchmarked, not just collected.

Patients & caregivers	Experience survey (Child-HCAHPS / Picker PPE-15 basis)	LIVE NOW
Staff	Safety culture (AHRQ HSOPS 2.0)	LIVE NOW
Visiting consultants	Physician engagement & satisfaction	TO COME
Payors / HMOs	Provider–payer relationship, both directions + claims analytics	TO COME
Referring clinicians	Referrer satisfaction (validated instruments + NPS)	TO COME



Two surveys are already live and collecting

Both are anonymous, take a few minutes, and open on any phone with no login. The more voices, the sharper the plan we bring back to the board.

Staff Safety Culture

For every member of the Haven team

consultforafrica.com/haven-audit.html

Tito will share the direct links and a QR code for printing.

Patient / Caregiver Experience

A QR card at the front / discharge desk

consultforafrica.com/haven-patient-survey.html

Tito will share the direct links and a QR code for printing.



What we will ask Haven for

A single secure, read-only folder does it. Twelve Week-1 items unlock the analysis immediately; the rest can follow. Partial records are fine; nothing here is a test.

- **Finance** Management accounts / P&L (12 mo), bank statements, trial balance, budget
- **Revenue & receivables** Revenue by service & payer, receivables ageing, price list / tariff
- **HMO / payer** HMO contracts & tariffs, claims register, denials, ageing per HMO
- **Pharmacy & stock** Full stock count with values, sales & purchase records, wastage
- **Procurement** Supplier terms & lead times, payables ageing
- **People & payroll** Full staff list, payroll / salary schedule, JDs & KPIs, commission structure
- **Clinical governance** Protocols, incident & M&M records, training, NICU criteria
- **Operations** Patient volumes (12 mo), occupancy, booking system, SOPs
- **Reporting & systems** Current management reports, KPIs, EMR / billing system access
- **Corporate** Licences (HEFAMAA), org chart, board minutes, insurance

The full itemised Information & Data Request accompanies this deck.



Two quick wins, before the audit even reports

1. A crash cart you can trust

A stocked, sealed, checklist-governed emergency standard with a shift-level check routine, the most basic safety assurance, in place from the very first week.

2. Cash starts moving

An immediate recovery push on the largest HMO receivables (Leadway, NEM) so working capital starts freeing up before the audit reports. The work begins to pay for itself.



Mobilise now, present to the board in week four

Week 1

Mobilise

Point person agreed, data request issued, surveys circulated, quick wins live, accounts reconciled

Week 2

On site

Interviews and observation across shifts, SOP walk-throughs, NICU readiness

Week 3

Deep-dives

Finance & working capital, procurement, HMO economics, reporting integrity; surveys close

Week 4

Board readout

Synthesis into a prioritised, costed plan, presented in a working session

WHAT HAPPENS NOW

The audit has begun. Here is week one.

- Tito agrees a point person with you, and we suggest the Head of Operations.
- The Information & Data Request goes across; the twelve Week-1 items unlock the analysis.
- The staff and patient surveys circulate; the crash-cart standard and receivables push go live.
- We reconcile the management accounts and confirm the on-site schedule for week two.

Tito Ipinmoye · Engagement Lead · Consult for Africa

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